



**POIEMA PROJECT**  
WHITEPAPER

# The First Twenty Years

*What young women carry between ages 10 and 30, and  
where Wellspring can lead.*

A whitepaper on adolescent and young adult women's health.

*Prepared for Wellspring Health Clinic.*

v1.1 // AUGUST 2026

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## Executive Summary

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Young women ages 10 to 30 carry a heavy, unaddressed set of hormonal and reproductive health issues through adolescence and early adulthood. Adults file severe period pain, heavy bleeding, irregular cycles, and mood disruption under normal teenage life. The real signal goes unread. The standard clinical response is hormonal birth control, prescribed to suppress the symptom without treating the cause underneath it, often without full informed consent.

This gap is also room to build something better. A clinic willing to investigate root causes, teach cycle literacy, and treat the mind and body as one system can serve a population that has waited a long time for real care. This document lays out what young women are living through, the misconceptions steering their care in the wrong direction, where Wellspring Health Clinic can lead, and the revenue case for leading first.

01

## What They're Living With

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*These patterns show up again and again, in the research and in the stories young women tell about their own bodies.*

### Severe Period Pain

Up to 90% of menstruators experience painful periods, the most common gynecologic condition there is. Up to 25% need regular medication or miss school and work because of it. Pain at that level is not biologically normal. Endometriosis causes most adolescent dysmenorrhea, and the average diagnosis takes 3.5 to 12 years.

### Heavy Bleeding

Heavy bleeding and anemia send teenagers to the emergency room on a regular basis. Up to a third of them carry an undiagnosed bleeding disorder underneath the visible symptom.

## **Irregular or Missing Cycles**

Functional Hypothalamic Amenorrhea shuts down ovulation in response to stress, under-eating, or over-exercising. It shows up often in young female athletes, where coaches and doctors typically reach for the pill without addressing the stress or the fuel deficit driving it.

02

# **The Misconceptions Getting in the Way**

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## **The 28-Day Cycle Myth**

Healthy adult cycles range from 25 to 35 days. Teen cycles run wider still, from 21 to 45 days, because the reproductive system takes up to 12 to 13 years to fully mature after a first period.

## **The Period Is a Vital Sign**

ACOG names the menstrual cycle a vital sign for adolescents. Estrogen and progesterone from regular ovulation build bone density, brain health, and heart health during the developing years. Suppressing ovulation early raises long-term risk for osteoporosis and cardiovascular disease.

## **What the Pill Actually Does**

The pill shuts down ovarian function and replaces natural hormones with synthetic ones. The result is a withdrawal bleed, a synthetic bleeding event distinct from ovulatory menstruation. Using it to treat irregular cycles, acne, or pain in teens masks conditions like PCOS or endometriosis, letting them progress underneath.

03

# **What's Working Against Them**

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## **A Medical Model Built to Suppress**

Conventional care defaults to shutting down ovarian function. Doctors prescribe the pill for acne, cramps, and irregular cycles as a first response, often without full informed consent about the tradeoffs. Doctors leave young women with Primary Ovarian Insufficiency, ovaries that stop functioning normally well before age 40, on

the same synthetic pill instead of the body-identical hormone replacement their bones and heart actually need.

### **Mental Health Risk from Hormonal Birth Control**

The female brain finishes developing around age 25, using natural estrogen and progesterone to build its neural pathways. Introducing synthetic hormones during adolescence disrupts that process. Studies show adolescents who start the pill have a 130% higher rate of depressive symptoms, with suicide attempt risk peaking among teens and first-time users. Most providers never run a screening like the PHQ-9 before writing that prescription, or after.

### **Nutrient Depletion from the Pill**

The pill depletes B12, folate, B6, B2, magnesium, and zinc, the exact nutrients neurotransmitter synthesis depends on. That depletion compounds mood issues and brain fog already in motion.

### **An Environment Working Against Their Biology**

Young women grow up exposed to a steady mix of endocrine-disrupting chemicals, xenoestrogens like BPA that mimic estrogen and create metabolic chaos in the body. High sugar diets, inadequate sleep, and constant social pressure dysregulate circadian rhythm and blood sugar, and hormonal health takes the hit.

### **The Post-Surgical Rehab Vacuum**

Hysterectomy and C-section rank among the most common surgeries performed on women in the United States, at any age. An athlete who sprains an ankle gets a physical therapy referral as standard practice. A woman recovering from a major pelvic surgery typically gets none. Untreated scar tissue and visceral adhesions bind the bladder, bowel, and uterus underneath the incision, driving chronic pelvic pain, bloating, and incontinence for years after the surgery itself is called a success.

04

## **Where Wellspring Can Lead**

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*Every gap above maps to a service line. Here is where Wellspring can build a specialty practice most clinics have not touched.*

### **Community and Cycle Education**

Wellspring can build mentorship and education programming for preteens and teens, a space where a girl learns to read her own cycle instead of hiding it. Cycle tracking

taught from the first period builds a health record most young women never get, and it gives Wellspring an ongoing point of contact with patients and their families.

## **A Different Standard for Pain**

Wellspring can train intake and clinical staff to treat disruptive period pain as a diagnostic signal that deserves real investigation. Checking for endometriosis, bleeding disorders, and hormonal imbalance becomes standard protocol from the first visit.

## **Pelvic Health and Manual Therapy**

Trauma-informed pelvic physical therapy, including protocols like Mercier Therapy, treats the physical root of pelvic pain and endometriosis symptoms directly. Restoring blood flow to the pelvis eases dysmenorrhea, corrects uterine malposition, and improves natural conception rates. Wellspring can lead with external, consent-based options and build trust before any internal exam, referring out for surgery or medication only when the body calls for it.

## **Restorative Hormone Care**

Restorative models like NaPro Technology use a woman's own cycle chart as the diagnostic tool, running hormone tests on the correct days of her cycle. Wellspring can build this into a root-cause approach to PCOS, adenomyosis, and endometriosis.

## **Body-Identical Hormone Restoration**

For patients who need actual hormone replacement rather than suppression, particularly those with Primary Ovarian Insufficiency, Wellspring can prescribe transdermal body-identical estradiol and oral micronized progesterone instead of a synthetic pill. Body-identical estradiol matches human physiology and protects developing bone and cardiovascular systems without the clotting risk carried by oral synthetic hormones. Progesterone is neuroprotective and anti-inflammatory. It offsets estrogen-driven tissue growth, the same mechanism that protects the endometrium in hormone therapy, and eases anxiety in some patients. This is a distinct prescribing service from the diagnostic work in Restorative Hormone Care above. It is a second, separate revenue line built on the same root-cause philosophy.

## **Post-Surgical Pelvic Rehabilitation**

Wellspring can build a standard rehabilitation pathway for every hysterectomy and C-section patient, the same way orthopedic surgery already comes with a physical therapy referral attached. Early manual therapy softens scar tissue before it fully sets, shortening a recovery timeline most patients are never told could be different. Few clinics have standardized this yet, and it reaches every age, not only the 10 to 30 bracket the rest of this document centers on.

## **Integrated Mental Health**

Wellspring can build mental health screening directly into hormonal care from day one, starting with a PHQ-9 before any hormonal prescription and again at follow-up. Endometriosis and chronic pelvic pain carry real emotional weight, and a nervous system stuck in fight-or-flight actively suppresses reproductive and hormonal function. Treating the mind and the cycle as one system sets Wellspring apart in a market that still treats them separately.

## **Nutrition Built Around Hormones**

Blood sugar balance is foundational to treating PCOS, heavy bleeding, and severe PMS. Protein, healthy fat, and fiber at every meal stabilize insulin and cortisol together. Circadian eating, front-loading calories earlier in the day, measurably lowers testosterone, insulin, and cortisol. Gut health plays its own role. The estrobolome, the gut bacteria that clear used estrogen from the body, needs support to keep spent estrogen moving out instead of recirculating. Wellspring can build nutrition counseling around all three principles as a core service line, handled in house.

**05**

# **The Business Case**

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Every service above is also a new revenue line. NaPro Technology and cycle charting, Mercier Therapy, body-identical hormone restoration, post-surgical pelvic rehabilitation, mental health screening, and hormone-supportive nutrition counseling: six distinct offerings. Several of them are typically cash-pay in this field, revenue that does not wait on an insurance reimbursement cycle.

Root-cause care keeps a patient coming back. A cycle chart needs review. A hormone protocol needs adjustment. A rehabilitation series runs for months. A birth control prescription ends the conversation at pickup. Restorative care extends it for years.

Few clinics combine these services under one roof. That gap is Wellspring's opening to lead, with a generation of patients who will be choosing a provider for the next ten years of their reproductive life. The opportunity and the mission are the same gap, looked at from two directions.

Young women ages 10 to 30 spend over a decade hearing that their symptoms are normal. The standard care model suppresses the symptom and moves on, at every

stage. Wellspring has room to build something else: care that investigates, treats the whole system, and earns the trust of a generation that has learned to expect less.

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